

Prior Authorization Request - Inpatient Admission

Health Plan

Humana Medicare Advantage. Plan ID: H5216-014-000. Request type: standard pre-service review.
Date of request: 2026-10-04

Member Information

Member: Raymond Okafor DOB: 1954-01-30 (age 72) Member ID: MBR-30502
Address: 2290 Lakeshore Blvd, Cleveland, OH 44108
Phone: (216) 555-0119 Sex: M
Primary insurance: Humana Medicare Advantage (primary). Secondary: none on file.

Requesting Provider

Dr. Priya Venkataraman, MD -- Cardiac Electrophysiology
NPI: 1830417562
Practice: Lakeshore Cardiovascular Specialists
Phone: (216) 555-0140 Fax: (216) 555-0141

Requested Service

Requested service: Elective inpatient admission, implantation of a transvenous implantable cardioverter defibrillator (ICD)
CPT 33249 ICD-10: I42.0 (Dilated cardiomyopathy); I50.22
Planned admit date: 2026-11-17
Level of care requested: INPATIENT
Place of service: Hospital, inpatient (acute care).

Submission Contact

Submitted through the provider portal by Tomas Ibarra, Authorization Coordinator.
Callback: (216) 555-0140. Supporting clinical documentation attached (7 pages including this request).

Cardiology Consultation Note

Date of service: 2026-09-09. Patient: Raymond Okafor Age: 72 Sex: M

HPI

72-year-old man with a non-ischemic dilated cardiomyopathy diagnosed last year. He was started on heart failure medications after the diagnosis. He walks his dog daily and can climb a flight of stairs; he notes slight breathlessness with brisk walking uphill but not with ordinary activity. No orthopnea. No syncope. No ICD discharges (no device). Symptoms put him in NYHA class II.

Ischemic work-up and events

Cardiac catheterization (see page 4) showed no obstructive coronary disease. He has not had a myocardial infarction, bypass surgery or stent at any time, certainly not within the last year.

PMH

Hypertension. Hyperlipidemia. Gout. Chronic kidney disease stage 2. Left rotator cuff repair 2017. Non-smoker. Occasional wine.

Exam

BP 122/76, HR 64, SpO2 98%, BMI 31. No JVD. Clear lungs. Regular rhythm, no gallop. No edema.

Assessment and plan

Non-ischemic dilated cardiomyopathy with HFrEF, NYHA II. His EF has improved on therapy. Continue GDMT. Discussed ICD candidacy with EP; formal shared decision making visit on 2026-09-18 (page 5). Surgical plan on page 6.

Echocardiogram Report - Most Recent

Study date: 2026-08-20. Type: complete transthoracic echocardiogram with contrast (Definity).

Patient: Raymond Okafor DOB: 1954-01-30 Interpreting: A. Nwosu, MD, FACC.

Comparison: no prior study available for comparison at the time of this interpretation.

Findings

Left ventricle mildly dilated, LVIDd 5.6 cm. Mild global hypokinesis, worse in the inferolateral wall. Overall left ventricular systolic function is mildly reduced, LVEF 42% (biplane Simpson, contrast-enhanced).

Normal LV wall thickness. Grade I diastolic dysfunction. Left atrium mildly dilated. RV normal size and function. Trivial MR and TR. No pericardial effusion. No thrombus.

Impression

Mild LV dilatation with mildly reduced systolic function, LVEF 42%.

Cardiac Catheterization and Hemodynamic Report

Date: 2025-11-18. Operator: J. Petrakis, MD. Access: right radial.

Patient: Raymond Okafor.

Coronary angiography

Left main normal. LAD with minimal luminal irregularities, no flow-limiting stenosis. LCx normal. RCA dominant, normal. No obstructive coronary artery disease.

Hemodynamics

LVEDP 20 mmHg. PCWP 18 mmHg. No gradient across the aortic valve. Right heart pressures mildly elevated.

Impression

Non-ischemic cardiomyopathy. No intervention performed.

Heart failure medications on file (clinic list, updated 2026-09-09)

Drug	Dose	Start date
sacubitril/valsartan	97/103 mg BID	2025-12-08
carvedilol	25 mg BID	2025-12-08
spironolactone	25 mg daily	2026-01-12
empagliflozin	10 mg daily	2026-01-12
amlodipine	5 mg daily	2018
atorvastatin	40 mg daily	2020
allopurinol	100 mg daily	2019

Total time on guideline-directed medical therapy: 9 months, adherence confirmed.

Shared Decision Making Visit - ICD

Date: 2026-09-18. Clinician: Dr. P. Venkataraman. Patient: Raymond Okafor; wife present.

Patient-facing evidence-based ICD decision aid (video and printed worksheet from the national heart failure decision-support program) was reviewed together at this visit. Benefits (lower risk of sudden cardiac death), risks (infection, shocks, lead failure, bleeding), alternatives (medical therapy alone, wearable defibrillator), and personal values were discussed for 30 minutes. The patient asked good questions and understood the content.

The patient wishes to proceed. Decision made jointly with the clinician, before scheduling the implant.

Electrophysiology Surgeon Operative Plan

Date: 2026-09-29. Surgeon: Dr. Priya Venkataraman, MD, electrophysiology. Patient: Raymond Okafor.

Planned procedure

Transvenous ICD implant, CPT 33249, left subclavian access.

Indication as submitted

Primary-prevention ICD for non-ischemic dilated cardiomyopathy, on a background of HFrEF, NYHA II. See imaging in this packet for the most current ventricular function.

Hospital course

Expected length of stay: 2 midnights for post-implant telemetry and device check.

Comorbidities

Heart failure, hypertension, hyperlipidemia, chronic kidney disease stage 2, gout.

Condition

Clinically stable, euvolemic, no conditions limiting survival, no severe neurologic impairment, no atrial fibrillation. No exclusion identified.

Outside Records - Echocardiogram (prior)

Records received from: Fairview Regional Medical Center, Cardiology Lab. Released 2026-09-02.

Study date: 2025-11-04. Patient: Raymond Okafor DOB: 1954-01-30

Indication: new dyspnea and elevated BNP, evaluate LV function.

Findings

Left ventricle moderately dilated (LVIDd 6.1 cm). Global hypokinesis. LVEF 30% by biplane Simpson. Moderate functional mitral regurgitation. LA moderately enlarged. RVSP 41 mmHg. No effusion.

Impression

Dilated cardiomyopathy with moderately to severely reduced LVEF (30%). Recommend ischemic evaluation and initiation of guideline-directed medical therapy.

Signed: Dr. E. Ramaswamy, 2025-11-04.

ECG, 2026-09-09 (this practice)

Sinus rhythm 62 bpm, PR 174 ms, QRS 96 ms, no pathologic Q waves, QTc 430 ms.